

FLORA WELLNESS

Complete Guide Series | Volume 04

The Hormone Symptom Decoder

What Every Symptom Is Telling You About
Your Hormones — And How to Fix It

From fatigue to acne to anxiety — decode the root cause

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SECTION 1: HOW TO USE THIS GUIDE

The Symptom-Hormone Connection

Every symptom your body produces is a message — a communication from your endocrine system that something in its environment needs to change. Instead of silencing symptoms with medication, the Hormone Symptom Decoder teaches you to read these messages and understand exactly what your body is asking for. Most hormonal symptoms have identifiable root causes, and when you identify and address those root causes, the symptoms resolve naturally.

Symptoms don't occur in isolation. The same root cause (say, chronic stress depleting progesterone) will express itself through a cluster of related symptoms: anxiety before the period, insomnia, spotting, PMS, and shortened cycles. When you see a symptom cluster rather than isolated symptoms, you're looking at a hormonal pattern — and patterns have protocols.

Use this guide as a reference: look up your most troubling symptom, read the full hormonal explanation, then check whether other symptoms in the same section also apply to you. This pattern-matching approach is far more useful than chasing individual symptoms, and it points you toward the targeted intervention that addresses the root cause rather than just the surface manifestation.

Important Medical Note

This guide is educational — it is not a replacement for medical evaluation. Any new, severe, or rapidly worsening symptoms require a proper clinical assessment. Many hormonal symptoms overlap with other medical conditions that require diagnosis and treatment. Use this guide as a starting framework, but always work with a qualified healthcare provider for diagnosis and personalized treatment.

SECTION 2: FATIGUE & ENERGY SYMPTOMS

Morning Fatigue Despite Adequate Sleep

Waking up exhausted after 8 or more hours of sleep is one of the most common and most misunderstood hormonal symptoms. It is not laziness, nor is it simply 'stress.' It is a physiological signal that your cortisol awakening response (CAR) is blunted. The CAR is a normal surge in cortisol that occurs within 30-60 minutes of waking, rising by 50-100% above baseline to provide energy, motivation, and mental clarity for the morning. When the adrenals are dysregulated — either overworked (Stage 1-2) or depleted (Stage 3) — this morning surge fails to occur properly.

Other hormones that cause morning fatigue: Low thyroid hormone (T3) reduces mitochondrial energy production, so cells literally cannot generate adequate ATP for energy. Low iron (ferritin below 50) impairs oxygen transport and mitochondrial function. Low vitamin D impairs mitochondrial function and cortisol regulation. B12 and folate deficiency affects energy metabolism at the cellular level.

The diagnostic key: does the fatigue improve during the day? If yes, this points to low morning cortisol and HPA dysregulation — you feel better as cortisol gradually rises through the day. If fatigue stays constant throughout the day with brief breaks, thyroid is more likely. If fatigue is worst after eating, investigate blood sugar and insulin. If fatigue worsens with any physical exertion (even mild), consider Stage 3 adrenal burnout.

Fatigue Pattern	Most Likely Hormone	Confirmatory Test	Fastest Intervention
Worst on waking, improves by noon	Blunted cortisol awakening response	4-point salivary cortisol or DUTCH	Morning sunlight + no phone 1 hour + adrenal breakfast
2-4pm daily crash	Post-lunch cortisol drop + blood sugar	Blood glucose log + cortisol testing	Protein-fat snack at 2pm; short walk; avoid refined carbs at lunch

Fatigue Pattern	Most Likely Hormone	Confirmatory Test	Fastest Intervention
Constant all day, sleep doesn't help	Thyroid (usually low T3)	Full thyroid panel including free T3	Optimize thyroid nutrition (selenium, iodine, iron); seek treatment
Crashes after meals	Reactive hypoglycemia, insulin	Glucose monitor post-meals	Protein + fat + fiber at every meal; no naked carbohydrates
Exercise wipes you out for days	Stage 3 adrenal burnout	DHEA-S, morning cortisol	Eliminate all intense exercise; rest + adrenal nutrition for 8 weeks
Only tired in luteal phase (days 18-28)	Low progesterone, magnesium deficiency	Mid-luteal progesterone blood test	Magnesium glycinate 400mg, Vitex, cycle syncing

The 2-4pm Energy Crash

The infamous afternoon energy crash — the slump between 2 and 4pm that sends millions of people to the coffee machine or the snack drawer — is not, as commonly believed, an inevitable part of the day. It is a cortisol dip that is significantly amplified by blood sugar instability, and it can be essentially eliminated with the right dietary approach.

Here's what happens: cortisol naturally peaks in the morning and has a secondary smaller peak around 1-2pm before declining in the afternoon. If you have healthy cortisol rhythm and stable blood sugar, this decline is gradual and imperceptible. But if you had a high-carbohydrate lunch (pasta, bread, rice, sugary drinks), your blood sugar spikes and then crashes 1-2 hours later, landing right at the same time as the cortisol dip. The combination creates a dramatic energy trough. The urge to reach for sugar or caffeine temporarily raises blood sugar or cortisol artificially, creating a vicious cycle.

The solution is simple in principle: eat a balanced lunch with substantial protein (minimum 25-30g), healthy fat, and fiber. This blunts the blood sugar spike and prevents the crash. A 10-minute walk after lunch further stabilizes glucose. If you still experience a crash despite good eating habits, get a 4-point cortisol test — if your cortisol is genuinely low at 2pm, this requires adrenal support rather than just

dietary changes.

SECTION 3: MOOD & MENTAL HEALTH SYMPTOMS

Anxiety That Worsens Before Your Period

Premenstrual anxiety is one of the most disruptive hormonal symptoms women experience, and it has a precise biochemical explanation. In the luteal phase (approximately days 15-28), progesterone rises and then falls dramatically in the days before menstruation. Progesterone converts to allopregnanolone, a neurosteroid that acts on GABA receptors — the same receptors targeted by anti-anxiety medications like benzodiazepines. Allopregnanolone is essentially your body's natural tranquilizer.

When progesterone is low — from chronic stress, anovulation, or perimenopause — allopregnanolone production is insufficient, and GABA-mediated calming is impaired. Additionally, the rapid drop of progesterone in the late luteal phase can cause a withdrawal effect similar to benzodiazepine discontinuation in sensitive women. This explains why premenstrual anxiety can be severe and feel completely out of proportion to any actual stressor.

What truly helps: Magnesium is the most direct intervention — it is a natural GABA agonist that enhances the same calming pathway as progesterone/allopregnanolone, at a dose of 300-400mg glycinate form in the evening. Vitex (chasteberry) supports progesterone production over 2-3 months and reduces the severity of premenstrual anxiety significantly in clinical trials. B6 as P5P supports neurotransmitter synthesis. And above all, stress reduction is critical — cortisol directly competes with progesterone and amplifies the anxiety response.

Depression and Low Mood Through the Cycle

Estrogen has profound effects on serotonin — it increases the expression of serotonin receptors in the brain and slows serotonin breakdown. When estrogen levels drop (premenstrually, perimenopausal, or post-partum), serotonin availability decreases, contributing to low mood, depression, and emotional sensitivity. This is why antidepressants that increase serotonin often temporarily help hormonal mood symptoms — they're working downstream on the same pathway. But they're not addressing the hormonal root cause.

The pattern of depression tied to your cycle is diagnostically useful. If mood is consistently low in the 7-10 days before your period and improves within 1-2 days of the period starting, this is classic PMDD or

severe PMS-D, driven by the progesterone/allopregnanolone mechanism and estrogen withdrawal. If low mood is constant but worse in certain cycle phases, look at overall estrogen levels and the broader hormonal picture. If depression appeared or worsened after pregnancy, postpartum hormonal crash (dramatic estrogen drop after delivery) is the primary driver.

Brain Fog — The Hormonal Explanation

Brain fog — the frustrating inability to think clearly, remember things, concentrate, or access words quickly — is one of the most alarming hormonal symptoms because it can feel like early dementia. In the vast majority of cases in reproductive-age women, it is not dementia. It is a reversible consequence of hormone imbalance.

Estrogen has direct effects on brain function: it increases blood flow to the prefrontal cortex, supports acetylcholine (the memory neurotransmitter), promotes synaptic plasticity, and protects neurons from inflammation. When estrogen fluctuates wildly (perimenopause) or declines significantly, brain function noticeably impairs. Thyroid hormone is equally critical — even mild hypothyroidism (free T3 below optimal) slows neurotransmitter production and impairs the brain's metabolic efficiency. And cortisol at chronically elevated levels directly damages the hippocampus — the brain's memory center.

SECTION 4: WEIGHT & METABOLISM SYMPTOMS

Weight Gain Despite Eating Well

Few symptoms frustrate women more than gaining weight — or being unable to lose it — despite genuinely eating well and exercising. This experience is real, and its cause is almost always hormonal. The standard advice (eat less, move more) ignores the fact that hormones directly regulate where the body stores fat, how efficiently metabolism functions, and whether the body is in a fat-burning or fat-storing mode.

Cortisol is the primary driver of weight gain 'despite doing everything right.' Chronically elevated cortisol triggers fat storage specifically in the abdomen (visceral fat), because these fat cells have the highest density of cortisol receptors. Cortisol also breaks down muscle tissue (reducing metabolic rate) and creates insulin resistance (increasing fat storage). A woman can eat 1400 calories in a calorie deficit but if cortisol is chronically elevated, the body will break down muscle for energy while hoarding fat.

Thyroid dysfunction is another major cause of weight gain that doesn't respond to diet. Low thyroid hormone reduces metabolic rate by 15-40% — a substantial reduction that cannot be overcome by diet alone. The weight typically distributes evenly (whole body, not just belly), the person feels cold, constipated, and fatigued, and the weight simply won't move regardless of caloric restriction. Standard thyroid treatment (if TSH is above 2.5-3.0 and symptomatic) often resolves this.

Weight Pattern	Hormone Driver	Key Diagnostic Clue	Primary Intervention
Belly fat specifically	Cortisol, insulin	Belly is disproportionately large; high stress; afternoon crashes	Cortisol reduction protocol; blood sugar stabilization
All-over weight gain, cold	Thyroid	Feel cold, constipated, hair loss, slow pulse	Full thyroid panel; selenium + iodine; possible medication

Weight Pattern	Hormone Driver	Key Diagnostic Clue	Primary Intervention
Weight gain in luteal phase	Water retention from estrogen-progesterone imbalance	Weight fluctuates 2-5 lbs with cycle; improves after period	Progesterone support; reduce sodium; magnesium; cruciferous veg
Can't lose despite calorie restriction	Cortisol, thyroid, under-eating causing stress response	Eating <1400 cal/day; under-eating raises cortisol further	Increase calories to maintenance; fix hormones first then optimize
Weight gain after 40	Estrogen decline reducing metabolic rate; insulin resistance	Perimenopausal symptoms present; weight gaining without diet change	Estrogen support; resistance training; insulin sensitivity protocol

Sugar and Carbohydrate Cravings — The Hormonal Root

Intense sugar and carb cravings are hormonal signals, not a lack of willpower. Understanding their source allows you to address the actual cause rather than white-knuckling it through willpower that will inevitably fail when hormones are driving the urge.

Afternoon sugar cravings (2-5pm)

Cause: Blood sugar crash from inadequate lunch protein/fat; secondary cortisol dip **Fix:** Balanced lunch with 30g protein; protein-fat snack at 2pm (apple + almond butter, cheese + crackers)

Pre-period carb and chocolate cravings (days 24-28)

Cause: Serotonin drops as estrogen falls; magnesium depleted in late luteal phase; brain craves quick serotonin from sugar **Fix:** Magnesium glycinate 400mg evening (often satisfies chocolate craving); 5-HTP 50mg; complex carbs instead of refined

Constant sugar cravings throughout day

Cause: Insulin resistance — cells not receiving glucose efficiently so brain demands more **Fix:** Protein at every meal; eliminate liquid sugar entirely; berberine; post-meal walks; inositol for PCOS

SECTION 5: CYCLE & REPRODUCTIVE SYMPTOMS

Heavy Periods — What Excess Blood Loss Means

A normal period involves losing 30-40ml of blood over 3-5 days. Heavy menstrual bleeding (menorrhagia) is defined clinically as more than 80ml per cycle — but practically, it's soaking through a regular pad or tampon in under 2 hours, passing clots larger than a quarter, or bleeding for more than 7 days. Heavy periods affect approximately 1 in 3 women at some point in their lives and are one of the leading causes of iron deficiency anemia.

The dominant hormonal cause of heavy periods is estrogen dominance — specifically, excess estrogen relative to progesterone during the cycle. Estrogen builds up the uterine lining (endometrium); progesterone stabilizes and contains it. When progesterone is insufficient, the lining builds excessively and sheds heavily and irregularly. The presence of clots indicates heavy blood flow that coagulation factors can't keep up with — not a separate condition.

Beyond hormonal causes: uterine fibroids (benign tumors driven by estrogen) and endometrial polyps are common structural causes of heavy periods that require medical evaluation. Thyroid dysfunction (both hypothyroid and hyperthyroid) can cause heavy periods through effects on clotting factors and prostaglandin production. A full evaluation for heavy periods should include: comprehensive hormonal panel, thyroid function, ultrasound to rule out structural causes, CBC to assess for anemia, and iron/ferritin levels.

The Period Blood Color Decoder

Color/Appearance	What It Indicates	Associated Hormones	Action Steps
Bright red, flowing easily	Normal healthy period — good estrogen levels, adequate flow	Healthy estrogen and progesterone balance	No action needed if not heavy
Dark brown or black blood (start or end of period)	Older blood that took longer to exit — often indicates slow flow	Low estrogen, low progesterone, or uterine congestion	Castor oil packs, uterine tonic herbs; assess estrogen levels

Color/Appearance	What It Indicates	Associated Hormones	Action Steps
Pink or light pink	Diluted blood — often indicates low estrogen	Low estrogen (under-eating, perimenopause, over-exercise)	Evaluate estrogen; ensure adequate caloric intake; rule out eating disorder
Dark red with large clots	Heavy flow with excess lining — estrogen dominance, fibroids	Excess estrogen, insufficient progesterone	Estrogen dominance protocol; pelvic ultrasound to rule out fibroids
Watery or thin	Low estrogen, thin endometrial lining	Low estrogen	Estrogen support; assess for perimenopause or eating disorder

SECTION 6: SKIN, HAIR & NAILS

Hormonal Acne — The Location Map

Where your acne appears on your face is highly diagnostic. The face-mapping approach, used in both Traditional Chinese Medicine and increasingly validated by modern dermatology, shows that specific facial regions correspond to specific organ and hormonal systems. While not perfectly precise, location patterns in combination with cycle timing and other symptoms can strongly suggest the hormonal driver.

Location	Hormone Driver	Why It Happens	Targeted Solution
Jawline and chin	Androgens (testosterone, DHT)	High androgen activity in lower face sebaceous glands; typically worsens premenstrually	Spearmint tea 2x daily, zinc 30mg, DIM, address PCOS/insulin if present
Cheeks (middle)	Liver congestion, gut dysbiosis	Liver processes hormones; gut dysbiosis drives systemic inflammation affecting skin	Liver detox protocol, gut microbiome support, reduce alcohol and processed food
Forehead	Gut and digestive system	Small intestine health reflects in forehead zone in face mapping	Gut health protocol, reduce sugar and processed food, digestive enzymes
Between eyebrows	Liver, alcohol, fatty foods	Direct liver correlation in face mapping; commonly worsens with alcohol	Liver detox, eliminate alcohol, reduce fat overload
Temples	Gallbladder, dehydration	Gallbladder processes bile from liver; dehydration impairs detox	Increase water, bitter foods (artichoke, dandelion), gallbladder support

Location	Hormone Driver	Why It Happens	Targeted Solution
Nose	Cardiovascular, blood pressure	Heart and circulation zone	Assess blood pressure; omega-3; anti-inflammatory diet

Hair Loss — Decoding the Pattern

Hair loss is one of the most distressing hormonal symptoms because it's visible, persistent, and slow to respond to treatment. The key is identifying the type of hair loss, because different patterns point to different hormonal causes and different solutions. Do not accept 'hormonal' as a complete answer — identify WHICH hormone and WHY.

Diffuse thinning all over scalp

Root Cause: Thyroid dysfunction, iron deficiency, general hormonal disruption, post-partum

Tests: Full thyroid panel, ferritin, B12, complete hormone panel **Fix:** Optimize ferritin to 70+; treat thyroid; postpartum hormone support

Loss of outer third of eyebrows

Root Cause: Thyroid — classic Hashimoto's sign **Tests:** TSH, Free T3, T4, TPO antibodies **Fix:** Thyroid treatment; selenium, iodine; potentially medication

Temple thinning (androgenic alopecia)

Root Cause: DHT sensitivity at follicles; elevated androgens **Tests:** Testosterone (free and total), DHEA-S, DHT if available **Fix:** Saw palmetto 320mg, zinc, address root androgen elevation

Sudden significant shedding (telogen effluvium)

Root Cause: High stress (cortisol spike), illness, surgery, crash diet, post-partum **Tests:** Hair pull test; check ferritin, thyroid; history for stressor 3 months prior **Fix:** Remove trigger if present; support ferritin, protein, and nutrients; patience

SECTION 7: SLEEP SYMPTOMS

Waking at 2-4am: The Liver and Cortisol Clock

According to both Traditional Chinese Medicine and modern circadian biology, consistent waking between 2-4am corresponds to the peak activity window of the liver. Modern science explains this: the liver performs most of its detoxification work between 1am and 3am, requiring significant energy. If blood sugar drops too low during this window (common in insulin-resistant individuals or those who haven't eaten enough before bed), the liver's detox work stalls and cortisol is released to raise blood sugar — waking you abruptly.

Hormonal causes of 2-4am waking: Low progesterone, as progesterone promotes deep sleep and its absence creates lighter, more disrupted sleep architecture. Perimenopause and its associated night sweats, driven by estrogen fluctuations causing vasodilation. High evening cortisol from HPA dysregulation (should be low by 10pm; if still elevated, prevents or disrupts sleep). Blood sugar dysregulation — the most commonly overlooked cause.

Practical solutions: Eat a small protein-fat snack before bed (1 tbsp almond butter or 1 oz cheese) if 2am waking appears to be blood sugar related. Take magnesium glycinate 300-400mg 30-60 minutes before bed to support progesterone action and GABA signalling. Get a 4-point salivary cortisol test to confirm whether elevated evening cortisol is contributing. Optimize blood sugar throughout the day — the bedtime snack addresses the symptom, but daytime blood sugar stability addresses the cause.

SECTION 8: DIGESTION & GUT SYMPTOMS

The Gut-Hormone Connection

The gut and hormone system are intimately and bidirectionally connected. The gut microbiome produces and metabolizes hormones, regulates estrogen through the estrobolome (see Guide 01), produces 90% of the body's serotonin (which affects both mood AND hormonal signalling), and contains more hormone receptors than any tissue other than reproductive organs.

When gut health is compromised — through dysbiosis, leaky gut, SIBO, inflammation, or constipation — hormones suffer in every direction. Estrogen is recirculated rather than eliminated. Serotonin production drops, affecting mood and cycle regulation. Inflammation from gut permeability triggers cortisol release. Nutrient malabsorption creates deficiencies in the micronutrients needed for hormone synthesis. Addressing gut health is therefore not separate from addressing hormones — it is integral to it.

Gut Symptom	Hormonal Connection	Root Cause	Intervention
Constipation (fewer than 1 BM/day)	Estrogen recirculation — most critical hormonal gut issue	Low thyroid, low magnesium, low water, insufficient fiber, sedentary	Magnesium citrate 200mg, 30g fiber/day, 2.5L water, walk daily; rule out thyroid
Diarrhea or loose stool before period	Elevated prostaglandins (PGE2) in luteal phase cause intestinal cramping	Excess prostaglandins from poor omega-6:omega-3 ratio, low magnesium	Increase omega-3 EPA, reduce omega-6 (seed oils), magnesium, avoid caffeine perimenstrually
Bloating after meals	Gut dysbiosis, SIBO, low stomach acid, food sensitivities	Often triggered by gluten, dairy, FODMAPs; can worsen with estrogen dominance	Digestive enzymes, elimination diet trial, probiotics, HCL supplementation if needed
Nausea (cyclical)	Estrogen fluctuations stimulate emesis center in brain	Peak estrogen (ovulation) or estrogen drop (perimenstrually)	Ginger tea, B6 25mg, small frequent meals; note cycle day pattern

Gut Symptom	Hormonal Connection	Root Cause	Intervention
GERD/heartburn	Cortisol reduces lower esophageal sphincter tone; progesterone relaxes esophagus	High cortisol and/or high progesterone in luteal phase	Reduce cortisol; smaller meals; elevate head of bed; digestive bitters before meals

SECTION 9: PERIMENOPAUSE SYMPTOM DECODER

Understanding the Perimenopausal Transition

Perimenopause — the transition leading to menopause — typically begins in a woman's early-to-mid 40s (though can start in late 30s) and lasts 4-10 years. During this time, ovarian function becomes irregular: some cycles are anovulatory (no ovulation, no progesterone), others have normal or even surging estrogen levels, creating a hormonal roller coaster that explains the extreme variability of perimenopause symptoms.

The hallmark of perimenopause is not simply 'low estrogen' — it is wild FLUCTUATIONS in estrogen that are often higher than at any other time in a woman's life, combined with significantly reduced progesterone. This estrogen dominance pattern in perimenopause is why many symptoms (breast tenderness, heavy periods, weight gain, mood swings) actually worsen before they improve as you approach menopause.

Symptom	Mechanism	Hormones Involved	Natural Interventions
Hot flashes	Estrogen fluctuations dysregulate hypothalamic thermostat; blood vessel dilation	Estrogen dropping; also cortisol and thyroid	Black cohosh 40mg; flaxseed 2 tbsp/day; sage tea; magnesium; avoid triggers (alcohol, sugar, spicy)
Night sweats	Same as hot flashes but occurring during sleep; disrupts sleep architecture	Estrogen fluctuations, cortisol	Cool bedroom (65°F); magnesium 400mg; estrogen support; BHRT with practitioner
Irregular cycles	Variable ovulation; sometimes estrogen surges, sometimes absent	FSH rising, LH erratic, progesterone variable	Vitex supports progesterone; track cycle to understand pattern; seed cycling

Symptom	Mechanism	Hormones Involved	Natural Interventions
Vaginal dryness	Declining estrogen reduces vaginal tissue thickness and lubrication	Estrogen decline	Topical estrogen cream (very safe, minimal systemic absorption); vitamin E oil; increase omega-3
Brain fog & memory changes	Estrogen supports acetylcholine, blood flow, and neuroprotection in brain	Estrogen and progesterone both declining	Omega-3 DHA prioritized; exercise (increases BDNF); phytoestrogens; BHRT consideration
Heart palpitations	Estrogen and magnesium deficiency affect cardiac electrical conduction	Estrogen drop; low magnesium	Magnesium glycinate 400mg; reduce caffeine; cardiovascular evaluation to rule out arrhythmia
Joint pain	Estrogen has anti-inflammatory effects; its decline increases inflammatory cytokines	Declining estrogen	Omega-3 4g/day; turmeric with black pepper; collagen 10g/day; maintain muscle mass
UTIs & vaginal infections	Declining estrogen alters vaginal microbiome and mucosal immunity	Estrogen	Topical estrogen; D-mannose 2g daily for UTI prevention; probiotics including vaginal strains

SECTION 10: THE MASTER SYMPTOM MATRIX

50 Symptoms Mapped to Hormones

Symptom	Primary Hormone	Secondary Hormone	Priority Action
Morning exhaustion	Cortisol (low CAR)	Thyroid	Morning light exposure + adrenal breakfast
Afternoon energy crash	Cortisol + blood sugar	Insulin	Balanced lunch + post-meal walk
Anxiety premenstrually	Progesterone (low allopregnanolone)	Cortisol	Magnesium glycinate + Vitex
Depression/low mood	Estrogen (serotonin connection)	Progesterone, thyroid	Estrogen support + omega-3 DHA
Brain fog	Thyroid (T3)	Estrogen, cortisol	Full thyroid panel + cortisol test
Heavy periods	Estrogen dominance	Thyroid, fibroids	DIM + Vitex + liver detox
Painful cramps	Prostaglandin excess	Magnesium deficiency	Omega-3 + magnesium + anti-inflammatory diet
Spotting before period	Progesterone deficiency	Estrogen dominance	Vitex + progesterone support
Irregular cycles	Multiple (stress, PCOS, thyroid, perimenopause)	Many	Track cycle + test hormones
Weight gain (belly)	Cortisol	Insulin	Cortisol protocol + blood sugar support

Symptom	Primary Hormone	Secondary Hormone	Priority Action
Can't lose weight	Thyroid	Cortisol, estrogen	Full thyroid panel + DUTCH test
Sugar cravings	Blood sugar, serotonin	Chromium, magnesium	Protein at every meal + chromium + magnesium
Insomnia/can't fall asleep	Cortisol (elevated)	Melatonin suppressed	Evening routine + magnesium + blue light blocking
Wake 2-4am	Blood sugar	Cortisol	Pre-bed protein snack + magnesium
Night sweats	Estrogen fluctuating	Cortisol	Cool bedroom + estrogen support
Low libido	Testosterone (low)	Estrogen, cortisol, thyroid	Zinc + testosterone testing + stress reduction
Vaginal dryness	Estrogen (low)	Testosterone	Topical estrogen + omega-3
Acne (jawline)	Androgens (DHT)	Insulin (PCOS)	Spearmint tea + zinc + inositol (if PCOS)
Hair loss (diffuse)	Thyroid	Ferritin, estrogen	Full thyroid + ferritin + hormone panel
Outer eyebrow thinning	Thyroid (Hashimoto's marker)	Thyroid	Thyroid antibody testing
Breast tenderness	Estrogen dominance	Iodine deficiency	DIM + iodine assessment + reduce caffeine

Symptom	Primary Hormone	Secondary Hormone	Priority Action
Hot flashes	Estrogen (fluctuating/declining)	Cortisol	Black cohosh + flaxseed + magnesium
Cold hands/feet	Thyroid	Iron	Thyroid panel + ferritin
Constipation	Thyroid	Magnesium, dehydration	Magnesium + fiber + water + thyroid check
Bloating	Gut dysbiosis	Estrogen dominance	Gut protocol + DIM
Nausea before period	Prostaglandins + estrogen		B6 + ginger + cycle awareness
Palpitations	Magnesium, estrogen	Thyroid, anxiety	Magnesium + estrogen support + cardiovascular eval
Joint pain	Estrogen (anti-inflammatory)	Omega-3 deficiency	Omega-3 + collagen + estrogen support
Mood swings (cyclical)	Progesterone + estrogen	Serotonin	Cycle syncing + magnesium + B6